

YOUR FULL REPORT

Are you carrying unprocessed trauma?

Significant distress deserves support

Score band: 36–45 points Relationships

What this report covers

A support-focused guide for significant trauma-related distress and the first steps toward greater safety.

Inside:

1. Stabilization before deep processing
2. Building a safe support map
3. Crisis help versus ongoing care

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Your result

Significant distress deserves support (36–45 points)

Many trauma-related responses appear active in your daily life. This quiz cannot diagnose PTSD, but the level of disruption you described is worth compassionate professional support.

Try this next

If you are in immediate danger or may harm yourself, contact local emergency or crisis support now. Otherwise, start with one safe, trauma-informed professional.

Your practical guide

A support-focused guide for significant trauma-related distress and the first steps toward greater safety.

1 Stabilization before deep processing

When distress is this active, stabilization comes before deep processing — this is the professional consensus, and it protects you. Processing trauma without stability is like surgery without anesthesia: technically possible, unnecessarily harrowing. Stabilization means: sleep as regular as you can make it, substances minimized (they borrow calm at high interest), daily grounding practice, and a predictable routine that tells your nervous system the world has order. If you are in therapy, tell your therapist you want to focus on stabilization skills first; a good trauma therapist will agree. The deep work will wait. Safety — internal and external — is the foundation everything else is built on.

2 Building a safe support map

Build a safe support map on paper: inner circle (1-2 people who know the real situation and can be called anytime), professional support (therapist, psychiatrist, support group, crisis line numbers saved in your phone), daily anchors (the routines, places, or practices that steady you), and emergency exits (where you would go and who you would call if things became unsafe). Share the map with one trusted person. Update it as things change. On your worst days, decision-making collapses — the map decides in advance, so you do not have to. This is not pessimism; it is the same reason buildings have fire exits. You hope not to need it, and you are safer for having it.

3 Crisis help versus ongoing care

Know the difference between crisis help and ongoing care, and use each for its purpose. Crisis resources — 988 (call or text, US), Crisis Text Line (text HOME to 741741), local mobile crisis teams — are for moments when you might act on harmful thoughts, feel unsafe, or cannot cope; they are immediate, free, and non-judgmental. Ongoing care — a trauma-informed therapist, psychiatrist if medication might help, support groups — is for the slow work of healing. One does not replace the other. If you are in immediate danger or may harm yourself, contact emergency or crisis support now, from this moment. Your life matters more than any plan, any stigma, any waitlist. Reach out today.